

Encounter Date: [REDACTED]

UUID: 60886A97-E046-4089-B86E-6498CC6F8648
TCGA-GV-A3QG-01A-PR

Redacted



SURGICAL PATHOLOGY (Order [REDACTED])

Patient Name [REDACTED]

Sex [REDACTED]

DOB [REDACTED]

Results

Specimen #: [REDACTED]
Submitting Physician: [REDACTED]

ICD-O3
carcinoma, urothelial, NOS
8120/3
Site: bladder, posterior wall
2-29-12 RP C67.4

FINAL DIAGNOSIS

- A. DESIGNATED LEFT DISTAL URETER, EXCISION:
SEGMENT OF URETER, NEGATIVE FOR MALIGNANCY.
- B. DESIGNATED RIGHT DISTAL URETER, EXCISION:
SEGMENT OF URETER, NEGATIVE FOR MALIGNANCY.
- C. LEFT EXTERNAL ILIAC LYMPH NODES, FIVE, EXCISION:
FIVE LYMPH NODES, NEGATIVE FOR TUMOR (0/5).
- D. LEFT OBTURATOR AND DEEP OBTURATOR LYMPH NODES #2, TWO, EXCISION:
TWO LYMPH NODES, NEGATIVE FOR TUMOR (0/2).
- E. LEFT COMMON ILIAC LYMPH NODE, ONE, EXCISION:
ONE LYMPH NODE, NEGATIVE FOR TUMOR (0/1).
- F. LEFT PARA-AORTIC LYMPH NODES, THREE, EXCISION:
THREE LYMPH NODES, NEGATIVE FOR TUMOR (0/3).
- G. PRESACRAL LYMPH NODES, SEVEN, EXCISION:
SEVEN LYMPH NODES, NEGATIVE FOR TUMOR (0/7).
- H. RIGHT EXTERNAL ILIAC LYMPH NODES, FIVE, EXCISION:
TWO OF FIVE LYMPH NODES POSITIVE FOR METASTATIC CARCINOMA (2/5).
COMMENT: The morphologic features are consistent with urothelial primary.
- I. RIGHT OBTURATOR AND DEEP OBTURATOR LYMPH NODES # 2, TWO, EXCISION:
TWO LYMPH NODES, NEGATIVE FOR TUMOR (0/2).
- J. RIGHT COMMON ILIAC LYMPH NODES, THREE, EXCISION:
ONE OF THREE LYMPH NODE POSITIVE FOR METASTATIC CARCINOMA (1/3).

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HIPAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Noted		
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	[Signature]	[Signature]

COMMENT: The morphologic features are consistent with urothelial primary.

K. RIGHT PARACAVAL LYMPH NODES, FOUR, EXCISION:

FOUR LYMPH NODES, NEGATIVE FOR TUMOR (0/4).

L. APPENDIX, APPENDECTOMY:

FIBROUS AND FIBROADIPOSE OBLITERATION OF APPENDICEAL LUMEN, NEGATIVE FOR TUMOR.

M. DESIGNATED LEFT DISTAL URETER, EXCISION.

SEGMENT OF URETER, NEGATIVE FOR TUMOR.

N. DESIGNATED RIGHT DISTAL URETER, EXCISION:

SEGMENT OF URETER, NEGATIVE FOR TUMOR.

O. LEFT OBTURATOR AND DEEP OBTURATOR LYMPH NODES # 1, TEN, EXCISION:

TEN LYMPH NODES, NEGATIVE FOR TUMOR (0/10).

P. RIGHT OBTURATOR AND DEEP OBTURATOR LYMPH NODES #1, FIVE, EXCISION:

FIVE LYMPH NODES, NEGATIVE FOR TUMOR (0/5).

Q. BLADDER AND PROSTATE, ROBOTIC LAPAROSCOPIC CYSTOPROSTATECTOMY:

URINARY BLADDER: HIGH GRADE INVASIVE UROTHELIAL CARCINOMA.

UROTHELIAL CARCINOMA EXTENDS TO THE POSTERIOR INKED SURGICAL MARGIN.

BILATERAL URETERAL AND URETHRAL SHAVE MARGINS ARE FREE OF TUMOR.

ANGIOLYMPHATIC INVASION IDENTIFIED.

PROSTATE GLAND: MULTIFOCAL WELL DIFFERENTIATED PROSTATIC ADENOCARCINOMA, GLEASON GRADE 6 (3 + 3).

PROSTATIC GLANDULAR ATROPHY AND NODULAR HYPERPLASIA.

SURGICAL MARGINS ARE FREE OF PROSTATIC ADENOCARCINOMA.

COMMENT: Invasive high grade urothelial carcinoma, 5.0 cm in maximum dimension, involves predominantly the posterior aspect of the bladder wall with extension to the right and left lateral walls. Prostatic adenocarcinoma greatest dimension, 4 mm and is multifocal involving bilateral prostate lobes.

Note: Ten specimen blocks requiring reprocessing prior to final microscopic examination.

[REDACTED]
COMMENT

URINARY BLADDER SYNOPTIC REPORT

SPECIMEN:

- Bladder.
- Other (specify): Prostate.

PROCEDURE:

- Radical cystoprostatectomy.

TUMOR SIZE:

- Greatest dimension: 5 cm.

HISTOLOGIC TYPE:

- Urothelial (transitional cell) carcinoma.

ASSOCIATED EPITHELIAL LESION:

- None identified.

HISTOLOGIC GRADE:

- Urothelial carcinoma (WHO
- High grade.

MICROSCOPIC TUMOR EXTENSION:

- Perivesical fat. MARGINS:
- Margins involved by invasive carcinoma.
- Specify margin: Posterior inked surgical margin.

LYMPH VASCULAR INVASION:

- Present.

PATHOLOGIC STAGING, (pTNM):

Primary tumor (pT)

- pT3a: Tumor invades perivesical tissue (microscopically).

Regional lymph nodes (pN):

- pN3

Number of lymph nodes examined:

Specify: 47, forty-seven

Specify number of lymph nodes involved (any size):

Specify: 3, three

Distant metastasis (pM):

- Not applicable.

RADICAL PROSTATECTOMY SYNOPTIC REPORT

PROCEDURE:

- Other (specify): Radical cystoprostatectomy.

PROSTATE SIZE:

- Weight: Cannot be assessed.
- Size: 10.5 x 4.3 cm.

LYMPH NODE SAMPLING:

- Pelvic lymph node dissection.

HISTOLOGIC TYPE:

- Adenocarcinoma (acinar, not otherwise specified).

HISTOLOGIC GRADE:

- Gleason pattern:
- Primary pattern:
- Grade 3

- Secondary pattern:
- Grade 3

- Tertiary pattern:
- Not applicable

- Total Gleason Score: 6

TUMOR QUANTITATION:

- Portion (percentage of prostate involved by tumor): 2% and/or tumor size (dominant nodule, if present):

- Greatest dimension: 4 mm

EXTRAPROSTATIC EXTENSION:

- Not identified.

SEMINAL VESICLE INVASION:

- Not identified.

MARGINS:

- Margins uninvolved by invasive carcinoma.

TREATMENT EFFECT ON CARCINOMA:

- Not identified.

LYMPH-VASCULAR INVASION:

- Not identified.

PATHOLOGIC STAGING, (pTNM):

Primary tumor (pT).

- pT2c: bilateral disease.

Regional lymph nodes (pN).

- pN0: no regional metastasis.

- Number of lymph nodes examined: 47, forty-seven

- Number involved: 0, zero [See comment].

Distant metastasis (pM):

- Not applicable.

COMMENT: Metastatic carcinoma in three lymph nodes sited in the Urinary Bladder template above is consistent with urothelial origin, not prostatic origin.

Final sign out performed at

(Electronic Signature)

SPECIMEN SUBMITTED

A: LEFT DISTAL URETER

B: RIGHT DISTAL URETER

C: LEFT EXTERNAL ILIAC

D: LEFT OBTURATOR AND DEEP OBTURATOR #2

E: LEFT COMMON ILIAC

F: LEFT PARA-AORTIC

G: PRE-SACRAL

H: RIGHT EXTERNAL ILIAC

I: RIGHT OBTURATOR AND DEEP OBTURATOR #2

J: RIGHT COMMON ILIAC

K: RIGHT PARACAVAL

L: APPENDIX

M: DISTAL LEFT URETER (CLIP ON MOST DISTAL)

N: DISTAL RIGHT URETER (CLIP ON MOST DISTAL)

O: LEFT OBTURATOR AND DEEP OBTURATOR #1

P: RIGHT OBTURATOR AND DEEP OBTURATOR #1

Q: BLADDER AND PROSTATE

CLINICAL DATA

Ca bladder

INTRAOPERATIVE CONSULT DIAGNOSIS

A. F.S.: Segment of ureter, negative for malignancy.

B. F.S.: Segment of ureter, negative for malignancy.

Q. F.S.: Urethral line of resection, negative for malignancy.

GROSS DESCRIPTION

A. Received fresh for frozen section consultation is a specimen labeled distal ureter. The specimen consists of a tubular segment of tan soft tissue measuring 1.0 cm. in length and 0.5 cm. in width. A pinpoint lumen is present. The specimen is totally submitted as follows:

- A1 - frozen section.
- A2 - remainder of specimen.

B. Received fresh for frozen section consultation labeled right distal ureter is a tubular segment of tan-pink soft tissue measuring 0.7 cm. in length and 0.6 cm. in width. Soft yellow adipose tissue is attached to one surface. The specimen is totally submitted as follows:

- B1 - frozen section.
- B2 - remaining soft tissue.

C. Received in fixative is an irregular lobulated, yellow-tan rubbery tissue segment 4.5 x 2 x 1.3 cm. On palpation a large nodule consistent with a lymph node is identified which measures 4.5 cm. in greatest dimension. The lymph node has fatty cut surfaces. Two full thickness sections of this node are sectioned and submitted separately in blocks 1 and 2. Two additional small nodules resembling lymph nodes are also identified. These lymph nodes are submitted in block 3. The remaining soft tissue is entirely submitted in blocks 4 and 5.

D. Received in fixative is a multiple floating segment of lobulated yellow-tan tissue which aggregates to approximately 2 x 2 cm. On section and on palpation three small areas of nodularity consistent with lymph nodes 0.2 to 0.5 cm. are identified. These lymph nodes are submitted in block 1. The remaining soft tissue is entirely submitted in block 2.

E. Received in fixative are floating segments of lobulated, yellow-tan tissue which aggregate to approximately 1.5 x 2 cm. On section definite lymph nodes are not identified. The specimen is entirely submitted in a single cassette as received.

F. Received in fixative are irregular segments of lobulated yellow-tan tissue measuring 3.5 x 2 cm. On section and on palpation three areas of nodularity resembling lymph nodes 1.0 to 1.5 cm. in greatest dimension are identified. These lymph nodes are entirely submitted in block 1. The remaining soft tissue is submitted in block 2.

G. Received in fixative are multiple irregular lobulated yellow-tan tissue segments aggregating to 5 x 3 cm. On section and on palpation small areas of nodularity resembling lymph nodes are identified measuring up to 1 cm. in greatest dimension. Sections of lymph nodes are removed and submitted in block 1. The remaining soft tissue is submitted in blocks 2-5.

H. Received in fixative are floating segments of lobulated yellow-tan tissue aggregating to approximately 4 x 2.5 cm. On section and on palpation areas of nodularity resembling lymph nodes are identified. The largest of which measures 2.4 cm. On section and on palpation areas of nodularity consistent with lymph nodes measuring up to 2.5 cm. in greatest dimension is identified. These lymph nodes are removed and submitted in blocks 1 and 2. The remaining soft tissue is submitted in blocks 3 and 4.

I. Received in fixative are multiple floating segments of lobulated, yellow-tan tissue which aggregate to approximately 2 x 2 cm. On section and on palpation definite nodules resembling lymph nodes are not identified

with certainty. The specimen is entirely submitted in a single cassette as received.

J. Received in fixative on a segment of telfa is an irregular segment of lobulated yellow-tan tissue measuring 3 x 2 cm. On section and on palpation an area of nodularity resembling lymph node is identified measuring 2.3 cm. in greatest dimension. It has a centrally fatty focally pink-tan cut surface. The lymph node is bisected and submitted in blocks 1 and 2. The remaining soft tissue is submitted in block 3.

K. Received in fixative are floating segments of lobulated, yellow-tan tissue aggregating to 2 x 3 cm. On section and on palpation definite areas of nodularity resembling lymph nodes are identified with certainty. The specimen is entirely submitted in a single cassette.

L. Received in fixative is an appendix measuring 4 cm. in length and 0.6 cm. in diameter. Attached periappendiceal adipose tissue measures 3 x 2 cm. A stapled suture line at the proximal margin of the appendix. The serosal surface on transection a pinpoint lumen is identified. No mass lesions are present. The appendix is sectioned and entirely submitted in a single cassette with a section adjacent to the stapled suture line painted with yellow ink.

M. Received in fixative is a tubular portion of grey tissue approximately 2 cm. in length and 0.7 cm. in diameter. A suture is present marking the distal margin of the specimen. A section adjacent to the clip representing the margin is submitted in a single cassette.

N. Received in fixative is a tubular portion of tan tissue approximately 0.8 cm. in length and 0.7 cm. in diameter. A clip is present on the distal margin of the specimen. A section adjacent to the clip representing the margin is submitted in a single cassette.

O. Received in fixative are multiple lobulated yellow-tan tissue segments aggregating to 4.5 x 3 cm. On section and on palpation areas of nodularity consistent with lymph nodes are identified measuring up to 3 cm. in greatest dimension. Sections of lymph nodes are submitted in blocks 1-3 with block 3 containing the largest lymph node which is bisected. The remaining soft tissue is entirely submitted in blocks 4-6.

P. Received in fixative is a lobulated, yellow-tan tissue aggregating to 4 x 2 cm. On section and on palpation areas of nodularity consistent with lymph nodes are identified. The lymph nodes are submitted in block 1. The remaining soft tissue is entirely submitted in blocks 2 and 3.

Q. Received in fixative is an en bloc resection consisting of prostate with attached seminal vesicles, peribladder and perivesicular adipose tissue. The entire specimen measures 14 x 12.5 x upwards of 7 cm. The prostate measures 10.5 x 4.3 cm. The right half of the prostate gland is painted yellow. The left of the prostate gland is painted black. The perineal surface posteriorly shows slight nodularity and granularity. The underlying bladder is somewhat firm. The resection surfaces of the perineal surface and the posterior aspect of the bladder is painted with blue ink. The prostatic urethral margin shows two plastic clips present. A section adjacent to the plastic clip representing the margin previously utilized for frozen section is submitted in block 1. After the clips are removed the shaved section adjacent to that utilized for frozen section is submitted for permanents in block 2. Each ureter is identified. The resection line of the ureter for this specimen and vas deferens on the right side is submitted in block 3. The resection line of the left ureter

for this specimen and vas deferens line of resection submitted in block 4. It is opened in surgery and demonstrates a large lesion present on the posterior wall measuring 5 x 3.9 cm. The lesion has a raised granular focally ulcerated appearance. It abuts the top of the trigone and is present close to the left ureteral orifice and within approximately 0.5 cm. of the right ureteral orifice. A section of the right ureteral orifice is submitted in block 5. A section through the left ureteral orifice is submitted in block 6. Sectioning through the middle of the trigone including the prostatic urethra is submitted in block 7. The tumor upon transection extends through the wall of the bladder and possibly to the blue inked surface. The bladder tumor measures upwards of 1.5 cm. in thickness. At the time of surgery in the fresh state is a portion of the tumor is taken for tissue procurement for banking. Representative sections of the tumor and posterior wall are submitted in full thickness in blocks 8-11. The remaining bladder mucosa away from the tumor is somewhat edematous and pink-tan and wrinkled. A representative section from the dome of the bladder is submitted in block 12. A representative section of the left lateral wall of the bladder including the edge of the tumor is submitted in block 13. A representative section of the right lateral wall of the bladder including the edge of the tumor is submitted in block 14. A section through the attached perivesicular adipose tissue demonstrates no grossly enlarged lymph nodes. Sectioning through the prostate demonstrates predominantly rubbery, pink-tan cut surfaces. No discrete areas of nodularity are identified within the gland. Representative sections from the right half of the prostate gland are submitted in blocks 15-17. Representative sections from the left half of the prostate gland are submitted in blocks 18-20. Upon transection of the seminal vesicles they are grossly unremarkable. A representative section from the right seminal vesicle is submitted in block 21. A representative section from the left seminal vesicle is submitted in block 22. Multiple representative sections.

Patient ID #: [REDACTED]
 Pt. Acct/Enc #: [REDACTED]
 DOB: [REDACTED] (Age: [REDACTED])
 Date of Report: [REDACTED]
 Date of Procedure: [REDACTED]
 Date of Receipt: [REDACTED]
 Submitted by: [REDACTED]
 Location: [REDACTED]
 Test performed by: [REDACTED]

Lab and Collection

SURGICAL PATHOLOGY (Order # [REDACTED] on [REDACTED] - Lab and Collection Information)

Result History

SURGICAL PATHOLOGY (Order # [REDACTED] for [REDACTED] Order Result History Report)

Result Information

Result Date and Time	Status	Provider Status
[REDACTED]	Final result	Reviewed

MyChart Status:

This result is currently not released to

[Display Full Result Report](#)

[Display Order Report](#)